

APO-MOMETASONE

Mometasone Furoate Aqueous Nasal Spray

50 mcg/metered spray

Corticosteroid

APO-MOMETASONE FUROATE AQUEOUS NASAL SPRAY 50MCG/METERED SPRAY is a white suspension that contains 50mcg of Mometasone furoate in each metered spray.

PHARMACODYNAMICS:

Mometasone furoate is a topical glucocorticosteroid with local anti-inflammatory properties at doses that are not systemically active. It is likely that much of the mechanism for the anti-allergic and anti-inflammatory effects of mometasone furoate lies in its ability to inhibit the release of mediators of allergic reactions. Mometasone furoate significantly inhibits the release of leukotrienes from leucocytes of allergic patients. In cell culture, mometasone furoate demonstrated high potency in inhibition of synthesis and release of IL-1, IL-5, IL-6 and TNF α . It is also a potent inhibitor of leukotriene production. In addition, it is an extremely potent inhibitor of the production of the Th2 cytokines, IL-4 and IL-5, from human CD4⁺ T-cells.

PHARMACOKINETICS:

Absorption: Mometasone furoate, administered as a nasal spray, has a systemic bioavailability of <1% in plasma, using a sensitive assay with a lower quantitation limit of 0.25 pg/mL.

Distribution: Not applicable as mometasone is poorly absorbed via the nasal route.

Biotransformation: The small amount that may be swallowed and absorbed undergoes extensive first-pass hepatic metabolism.

Elimination: Absorbed mometasone furoate is extensively metabolized and the metabolites are excreted in urine and bile.

INDICATIONS AND CLINICAL USE: APO-MOMETASONE Aqueous Nasal Spray is indicated for use in adults, adolescents and children between the ages of 3 and 11 years to treat the symptoms of seasonal or perennial rhinitis. In patients who have a history of moderate to severe symptoms of seasonal allergic rhinitis, prophylactic treatment with APO-MOMETASONE is recommended two to four weeks prior to the anticipated start of the pollen season.

APO-MOMETASONE Nasal Spray is also indicated for the treatment of nasal polyps in patients 18 years of age and older.

APO-MOMETASONE Aqueous Nasal Spray is indicated for the treatment of symptoms associated with acute rhinosinusitis in patients 12 years of age and older without signs or symptoms of severe bacterial infection.

RECOMMENDED DOSE AND DOSAGE ADJUSTMENT: Shake container well before each use.

Seasonal allergic or perennial rhinitis: Adults (including geriatric patients) and adolescents: The usual recommended dose for prophylaxis and treatment is two sprays (50 mcg/spray) in each nostril once daily (total dose 200 mcg). Once symptoms are controlled, dose reduction to one spray in each nostril (total dose 100 mcg) may be effective for maintenance.

If symptoms are inadequately controlled, the dose may be increased to a maximum daily dose of four sprays in each nostril once daily (total dose 400 mcg). Dose reduction is recommended following control of symptoms.

Clinically significant onset of action occurs as early as 12 hours after the first dose.

Children between the ages of 3 and 11 years: The usual recommended dose is one spray (50 mcg/spray) in each nostril once daily (total dose 100 mcg).

Administration to young children should be aided by an adult.

Nasal polypsis: Adults (including geriatric patients) and adolescents 18 years of age and older: The usual recommended dose for polypsis is two sprays (50 mcg/spray) in each nostril twice daily (total daily dose of 400 mcg). Once symptoms are adequately controlled, dose reduction to two sprays in each nostril once daily (total daily dose 200 mcg) is recommended.

Acute Rhinosinusitis: Adults (including geriatric patients) and adolescents 12 years of age or older: The usual recommended dose for acute rhinosinusitis is two actuations (50 mcg/spray) in each nostril twice daily (total daily dose of 400 mcg).

If no improvement is seen after 15 days of twice daily administration, alternative therapies should be considered. If symptoms worsen during treatment, the patient should be advised to consult their physician.

DOSAGE AND ADMINISTRATION: After initial priming of the APO-MOMETASONE Aqueous Nasal pump (10 actuations, until a uniform spray is observed), each actuation delivers approximately 100 mcg of mometasone furoate suspension, containing mometasone furoate monohydrate equivalent to 50 micrograms mometasone furoate. If the spray pump has not been used for 14 days or longer, it should be primed with 2 actuations, until a uniform spray is observed, before next use.

MODE OF ADMINISTRATION: Nasal Spray

CONTRAINDICATIONS: Hypersensitivity to the active substance, mometasone furoate, or to any of the excipients used in the product.

APO-MOMETASONE should not be used in the presence of untreated localised infection involving the nasal mucosa, such as herpes simplex. Because of the inhibitory effect of corticosteroids on wound healing, patients who have experienced recent nasal surgery or trauma should not use a nasal corticosteroid until healing has occurred.

WARNINGS AND PRECAUTIONS:

Immunosuppression

APO-MOMETASONE should be used with caution, if at all, in patients with active

or quiescent tuberculous infections of the respiratory tract, or in untreated fungal, bacterial, or systemic viral infections.

Patients receiving corticosteroids who are potentially immunosuppressed should be warned of the risk of exposure to certain infections (e.g., chickenpox, measles) and of the importance of obtaining medical advice if such exposure occurs.

Local Nasal Effects

Patients using APO-MOMETASONE over several months or longer should be examined periodically for possible changes in the nasal mucosa. If localised fungal infection of the nose or pharynx develops, discontinuance of APO-MOMETASONE therapy or appropriate treatment may be required. Persistence of nasopharyngeal irritation may be an indication for discontinuing APO-MOMETASONE.

APO-MOMETASONE is not recommended in case of nasal septum perforation.

APO-MOMETASONE contains benzalkonium chloride which may cause nasal irritation.

Systemic Effects of Corticosteroids

Systemic effects of nasal corticosteroids may occur, particularly at high doses prescribed for prolonged periods. These effects are much less likely to occur than with oral corticosteroids and may vary in individual patients and between different corticosteroid preparations. Potential systemic effects may include Cushing's syndrome, Cushingoid features, adrenal suppression, growth retardation in children and adolescents, cataract, glaucoma and more rarely, a range of psychological or behavioural effects including psychomotor hyperactivity, sleep disorders, anxiety, depression or aggression (particularly in children).

Patients who are transferred from long-term administration of systemically active corticosteroids to APO-MOMETASONE require careful attention. Systemic corticosteroid withdrawal in such patients may result in adrenal insufficiency for a number of months until recovery of HPA axis function. If these patients exhibit signs and symptoms of adrenal insufficiency or symptoms of withdrawal (e.g., joint and/or muscular pain, lassitude, and depression initially) despite relief from nasal symptoms, systemic corticosteroid administration should be resumed and other modes of therapy and appropriate measures instituted. Such transfer may also unmask pre-existing allergic conditions, such as allergic conjunctivitis and eczema, previously suppressed by systemic corticosteroid therapy.

Treatment with higher than recommended doses may result in clinically significant adrenal suppression. If there is evidence for higher than recommended doses being used, then additional systemic corticosteroid cover should be considered during periods of stress or elective surgery.

Effect on Growth in Paediatric Population

It is recommended that the height of children receiving prolonged treatment with nasal corticosteroids is regularly monitored. If growth is slowed, therapy should be reviewed with the aim of reducing the dose of nasal corticosteroid if possible, to the lowest dose at which effective control of symptoms is maintained. In addition, consideration should be given to referring the patient to a paediatric specialist.

Non-nasal Symptoms

Although APO-MOMETASONE will control the nasal symptoms in most patients, the concomitant use of appropriate additional therapy may provide additional relief of other symptoms, particularly ocular symptoms.

DRUG INTERACTIONS:

Co-treatment with CYP3A inhibitors, including cobicistat-containing products, is expected to increase the risk of systemic side-effects. The combination should be avoided unless the benefit outweighs the increased risk of systemic corticosteroid side-effects, in which case patients should be monitored for systemic corticosteroid side-effects.

PREGNANCY AND LACTATION:

Pregnancy: There are no or limited amount of data from the use of mometasone furoate in pregnant women. As with other nasal corticosteroid preparations, APO-MOMETASONE should not be used in pregnancy unless the potential benefit to the mother justifies any potential risk to the mother, foetus or infant. Infants born of mothers who received corticosteroids during pregnancy should be observed carefully for hypoadrenalism.

Lactation: It is unknown whether mometasone furoate is excreted in human milk. As with other nasal corticosteroid preparations, a decision must be made whether to discontinue or to discontinue/abstain from APO-MOMETASONE therapy taking into account the benefit of breast feeding for the child and the benefit of therapy for the woman.

ADVERSE REACTIONS:

Summary of the safety profile

Systemic effects of nasal corticosteroids may occur, particularly when prescribed at high doses for prolonged periods.

Tabulated list of adverse reactions

Adverse reactions are listed according to MedDRA primary system organ class. Within each system organ class, adverse reactions are ranked by frequency. Frequencies were defined as follows: Very common; common; uncommon. The frequency of post-marketing adverse events are considered as "not known (cannot be estimated from the available data)".

Table 1: Treatment-related adverse reactions reported by system organ class and frequency			
	Very common	Common	Not known
Infections and		Pharyngitis	

infestations		Upper respiratory tract infection [†]	
Immune system disorders			Hypersensitivity including anaphylactic reactions, angioedema, bronchospasm, and dyspnoea
Nervous system disorders		Headache	
Eye disorders			Glaucoma increased intraocular pressure Cataracts
Respiratory, thoracic and mediastinal disorders	Epistaxis*	Epistaxis Nasal burning Nasal irritation Nasal ulceration	Nasal septum perforation
Gastrointestinal disorders		Throat irritation*	Disturbances of taste and smell

*recorded for twice daily dosing for nasal polyposis

[†]recorded at uncommon frequency for twice daily dosing for nasal polyposis

SYMPTOMS AND TREATMENT OF OVERDOSAGE:

Symptoms

Inhalation or oral administration of excessive doses of corticosteroids may lead to suppression of HPA axis function.

Management

Because the systemic bioavailability of APO-MOMETASONE is <1%, overdose is unlikely to require any therapy other than observation, followed by initiation of the appropriate prescribed dosage.

DOSAGE FORM AND PACKAGING AVAILABLE: APO-MOMETASONE comes in a nasal spray device which delivers 140 sprays. Each spray delivers an unscented mist, containing the equivalent of 50 mcg of Mometasone furoate.

STORAGE CONDITIONS: Store below 30°C and protect from light. Do not freeze.

MANUFACTURER: Apotex Inc. (Richmond Hill), 380 Elgin Mills Road East, Richmond Hill, Ontario, Canada L4C 5H2.

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